Document type	**SOP (Standard Operating Procedure)**				
Intended for	Chair, Secretary and members of the Local Bioethics Committee of HemCenter LLP				
Nomenclature code	I-05	**Number**	Issl-3-9	**Revision**	**001**
Title	**REVIEW OF AMENDMENTS TO RESEARCH DOCUMENTS**				
Developed by	Business Development Director		Volyanskaya N.V.		15 May 2023
	Project Manager		Graf M.A.		15 May 2023
Agreed by	Medical Director		Klodzinskiy A.A.		29 May 2023
	Head of Patient Support and Internal Review Service		Demkina O.V.		29 May 2023
Approved	By Order of the Director General of HemCenter LLP No. I-02-52 dated 2 June 2023				
Implemented					
Revision policy	Upon changes in regulatory legal acts and/or operational need				

Purpose

To establish the procedure for considering amendments made to a previously approved study document package/ study protocol

Responsibilities

The Secretary of the Local Bioethics Committee of HemCenter LLP (LEC) is responsible for handling amendments to the study document package/ study protocol

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No. Terms and definitions

a. **Biological equivalence** (hereinafter – bioequivalence) – the absence of significant differences in the rate and extent to which the active substance or active moiety of pharmaceutical equivalents or pharmaceutical alternatives becomes available at the site of its action when administered at the same molar dose under similar conditions in an appropriately designed study

a. **Investigator's Brochure** - a compilation of the results of the clinical and preclinical (non-clinical) study of the investigational product that are relevant to its study in humans.

b. **State expert organization in the field of circulation of medicinal products and medical devices** (hereinafter – the expert organization) – a state monopoly entity carrying out production and economic activities in the field of healthcare to ensure the safety, efficacy and quality of medicinal products and medical devices.

c. **Study design** - the overall plan of the study, a description of the manner in which the study is conducted depending on the selection and formation of groups of study subjects and the blinding (masking) of data.

d. **Contract** - a dated and signed agreement between two or more parties that sets out arrangements concerning the allocation of the scope of work and responsibilities in the conduct of a study and, where necessary, financial matters. The study protocol should serve as the basis of the contract.

e. **Case report form** - a document on paper, electronic or optical media designed to record all the information required by the protocol to be reported to the sponsor on each study subject.

f. **Interventional study** – a study involving a human as a study subject, in which the investigator-physician, on the basis of an interventional clinical trial protocol that complies with the procedure for conducting clinical trials, assigns a specific intervention to the study subjects.

g. **Informed consent** - a procedure by which a study subject or his or her legal representative voluntarily confirms consent to participate in a particular study after having been informed of all aspects of the study that are relevant to the decision. Written informed consent is documented using the form approved by the authorized body.

h. **Bioequivalence study** – a comparative study of bioavailability aimed at establishing pharmacokinetic equivalence between the test product and the comparator product.

i. **Investigator** - an individual responsible for the conduct of a clinical trial at the clinical sites of HemCenter LLP, a member of the research team.

Where a study is conducted at a clinical site by a group of investigators, a **Principal Investigator** - the head of the research team - is designated. The Principal Investigator is selected by the members of the Clinical Project Office and approved by the Director General of HemCenter LLP. The Order appointing the members of the research team (investigators) is approved by the directors of the branches and the division of HemCenter LLP

j. **Investigational product** - a pharmaceutical form of an active substance or placebo being tested or used as a control in a clinical trial, including an authorised medicinal product where it is used in a way different from the approved one, or when used for a new indication or to gain further information about an approved indication

k. **Ethics committee** - a consultative and advisory body established at the national level (the Central Bioethics Commission) and at the level of a medical organization, composed of specialists in healthcare and science and representatives of public organizations, which protects the rights, safety and well-being of study subjects and investigators and carries out the ethical and moral-legal assessment of clinical trial materials.

Local Bioethics Committee (LEC) of HemCenter LLP - an independent body that protects the rights, safety and well-being of patients, trial subjects, investigators and healthcare workers and carries out the ethical and moral-legal assessment of materials related to medical and scientific activities.

l. **Clinical site** - a healthcare organization that conducts clinical trials.

m. **Clinical trial** - a study involving a human as a subject, conducted to identify or confirm the safety and efficacy of means, methods and technologies for the prevention, diagnosis and treatment of diseases.

n. **Contract research organization** (CRO, Contract research organization) — an individual/organization that, under a contract with the sponsor, performs one or more of the sponsor's functions related to the conduct of a clinical trial.

o. **Confidentiality** - keeping information belonging to the sponsor, or information that makes it possible to identify a study subject, secret from unauthorized persons. Materials containing restricted information.

p. **Medicinal product** – a product that is or contains a substance or combination of substances that comes into contact with the human body and is intended for the treatment or prevention of human diseases, or for restoring, correcting or modifying human physiological functions by exerting a pharmacological, immunological or metabolic action, or for the diagnosis of human diseases and conditions.

q. **Study materials** – a set of documents submitted by the sponsor of clinical trials to the clinical site, which includes the authorization of the authorized body in the field of healthcare of the Republic of Kazakhstan to conduct the given clinical trial and to designate those responsible for its conduct in accordance with the procedure approved in the Republic of Kazakhstan; the clinical trial protocol; the Investigator's Brochure; explanatory documents on the specifics of the use of new medical technologies (where used) and other documents, as well as the draft clinical trial contract

r. **Medical record** — a medical document in which the attending physicians record the patient's medical history (inpatient or outpatient) and the treatment prescribed to the patient, in electronic or paper form.

s. **Multicenter clinical trial** - a clinical trial conducted according to a single protocol at more than one study center and by more than one investigator.

t. **Clinical trial monitor** – an individual appointed by the sponsor or contract research organization who works together with the coordinating investigator or coordinating committee and oversees the conduct of the clinical

trial in accordance with the protocol, assesses the degree of its implementation, and assists the investigator in the analysis, interpretation and extrapolation of data.

u. **Monitoring** - the procedure of overseeing the progress of a biomedical study and ensuring that it is conducted, the data are collected and the study results are reported in accordance with the protocol, plan, programme, standard operating procedures and Good Clinical Practice.

v. **Good Clinical Practice (GCP)** - a standard for the design, organization, conduct, monitoring, auditing and recording of clinical trials, as well as for the analysis and reporting of their results, that provides assurance that the data obtained and the results reported are credible and accurate, and that the rights, health and confidentiality of study subjects are protected.

w. **Adverse incident** – any malfunction and/or deterioration in the characteristics or performance of a medical device, or any inadequacy or inaccuracy in the accompanying information (documentation) for a medical device, or side effects or an adverse reaction not specified in the instructions for use or the operating manual, which directly or indirectly led, or might have led, to the death or serious deterioration in the state of health of users or third parties.

x. **Adverse event** – any untoward change in the state of health of a patient or clinical trial (study) subject administered a medicinal (investigational) product, regardless of whether it has a causal relationship with its use.

y. **Impartial witness** - an individual not involved in the conduct of the clinical trial, - who assists the study subject, including by reading out the written materials supplied to him or her.

z. **Non-interventional clinical trial** – a study that is conducted after the state registration of a medicinal product or medical device and is prescribed within the framework of medical practice.

aa. **Original medical records** – see the term "source documents".

bb **Essential documents** – documents which collectively or individually permit evaluation of the conduct of a study and the quality of the data produced. These documents serve to demonstrate the compliance of the investigator, sponsor and monitor with the standards of Good Clinical Practice (GCP) and with regulatory requirements.

cc. **Clinical trial (study) report** - a document containing a description of a clinical trial of a therapeutic, prophylactic or diagnostic agent involving a human as a subject, which integrates the clinical and statistical descriptions, the presentation of data and their analysis; and long-term effects, including adverse reactions to the investigational medicinal product or medical device.

dd **Source documents** – all information contained in original medical records and certified copies thereof that describes the results of clinical observations, examinations and other activities and makes it possible to reconstruct and evaluate the course of the clinical trial. Source data are contained in source documents (originals or certified copies thereof).

ee. **Comparator product** – an authorised medicinal product or placebo used as a control in a clinical trial.

ff. **Clinical trial protocol** - a document that describes the objectives, design, methodology, statistical considerations and organization of a study.

gg. **Serious adverse event and/or serious adverse reaction** - an adverse reaction that results in death, is life-threatening, requires hospitalization of the patient or prolongation of existing hospitalization, results in persistent or significant incapacity or disability, or in congenital anomalies or birth defects, or requires medical intervention to prevent the development of the listed conditions

hh **Protocol synopsis** - a brief summary of the clinical trial protocol.

ii. **Clinical trial sponsor** – an individual or legal entity responsible for the initiation, organization and financing of a clinical trial.

jj. **Standard operating procedures** – detailed written instructions designed to achieve uniformity in the performance of a specific activity.

kk. **Study subject** - an individual who participates in a clinical trial either as part of the group receiving the investigational product or as part of the control group.

ll. **Authorized body** in the field of healthcare – the Ministry of Health of the Republic of Kazakhstan.

mm**Clinical trial phase:**

the first phase of a clinical trial – the first testing of a medicinal product, conducted in healthy volunteers or, where the high toxicity of the investigational product makes it unethical to conduct the study in healthy volunteers, in patients suffering from the relevant disease, in order to establish tolerability, safety, the presence of a therapeutic effect, and pharmacokinetic and pharmacodynamic characteristics;

the second phase of a clinical trial – a study aimed at assessing the efficacy and short-term safety of the investigational medicinal product in patients with a specific disease, as well as demonstrating the clinical efficacy of the medicinal product and determining the therapeutic dose level when tested in a group of patients;

phase four of a clinical trial – a study of a medicinal product conducted after its registration and placing on the market in order to confirm its therapeutic efficacy and the strategy for its further use, as well as to obtain additional information on the range and frequency of adverse effects and on the interaction of the investigational medicinal product with other medicinal products;

phase three of a clinical trial – a study conducted in large groups of patients of various ages and with various concomitant conditions in order to examine all aspects of treatment, including assessment of the risk: benefit ratio.

No. Abbreviations

a. **IB** – Investigator's Brochure
b. **PI** – Principal Investigator
c. **RT** – research team
d. **MD** – medical device
e. **IP** – investigational product
f. **CRF** – case report form
g. **IC** – informed consent
h. **CT** – clinical trial
i. **CTC** – clinical trial coordinator
j. **LEC** – Local Bioethics Committee of HemCenter LLP
k. **MoH RK** – Ministry of Health of the Republic of Kazakhstan
l. **AE** – adverse event
m. **ED** – essential documents
n. **RK** – Republic of Kazakhstan
o. **SAR –** serious adverse reaction
p. **SAE –** serious adverse event
q. **SOP**– standard operating procedure
r. **LLP** – limited liability partnership
s. **the Center** – HemCenter LLP
t. **CBC** – Central Bioethics Commission of the Ministry of Health of the Republic of Kazakhstan
u. **ҚР ДСМ** – abbreviation used when assigning numbers to regulatory legal acts of the RK (Kazakh)
v. **CRO** – contract research organization
w. **GCP** – Good Clinical Practice
x. **ICH-GCP** – International Conference on Harmonisation, the international conference on the harmonisation of GCP
y. **CIOMS** – Council for International Organizations of Medical Sciences

z. **EMEA –** European Medicines Agency (EMA), the agency for the evaluation of medicinal products for compliance with the requirements set out in the European Pharmacopoeia

aa. **FDA –** Food and Drug Administration of the USA (United States of America)

bb **UNESCO –** United Nations Educational, Scientific and Cultural Organization, a specialised agency of the United Nations for education, science and culture.

cc. **WHO** – World Health Organization

No. Procedure

1. Investigators make amendments to the study document package as necessary.

Amendments may be submitted either for expedited review (conducted by the LEC Chair or members) or for full review of the amendments by the LEC.

2. **Amendment document package** The applicant submits the amendment document package to the LEC Secretariat. The amendment document package includes:

1) a request from the applicant (Principal Investigator/ project leader) regarding the need to amend the document package previously submitted and approved by the LEC. The request should:

state/describe the amendment

state the reason for the amendment;

state all discrepancies from the original;

state the expected effects after the amendment is made.

2) the initial amendment submission form (Appendix 1)

3) the amended protocol and the relevant documents. All changes or additions must be underlined or highlighted.

3 **Notification of the Chair** The LEC Secretary notifies the Chair of receipt of the amendment document package orally or in writing. The amendment request is sent to the LEC Chair within 1 (one) working day of the date of receipt.

The LEC Secretary must keep the "Sent" and "Received" correspondence concerning notification of the Chair in the correspondence section of the study file.

4 **Determining the review status (full or expedited)** After reviewing the materials, the LEC Chair determines the review status - expedited or full.

The expedited review procedure is carried out in accordance with the standard operating procedure "Expedited Review".

The full review procedure is carried out in accordance with the standard operating procedure "Initial Review of Study Documentation (Applications, Study Protocols)".

Amendments which, in the opinion of the LEC Chair, increase the risk to study participants, such as changes to the study design, include, but are not limited to, the following changes:

additional treatment or its reduction;

any changes to the inclusion/exclusion criteria;

changes in the route of administration of the product, e.g. from oral to intravenous; significant changes in the number of participants (if the total number of participants is fewer than 20 subjects, a change of 5 persons is significant; if more than 20, a change of 20% is significant. Reduction: if a reduction in the number of participants alters the characteristics of the study, it is significant);

a significant decrease or increase in the dosage of the product.

If the Chair decides that the protocol requires approval at an LEC meeting, he/she records this decision on the checklist, and signs and dates the form. The Secretary places the amendment request on the agenda of the next LEC meeting.

Each LEC member receives the following documents:

documents for the examination of the amendments, giving a full picture of the nature of each change; the required changes to the informed consent form, where necessary. If the amendments were received immediately before the LEC meeting, the Chair may decide to consider them at a full LEC meeting even if they could be reviewed under the expedited procedure.

5 **Amendment review process** The amendment review procedure is carried out in accordance with the standard operating procedure "Study Evaluation".

The recommendation of the LEC members regarding changes to the protocol and/or the informed consent form is recorded in the LEC meeting minutes as "Changes made by the LEC" and will be communicated to the applicant.

The LEC Chair conducts a vote on the proposed amendments: to approve the amendments to the protocol without changes to the informed consent form; to require changes to the documents relating to the proposed amendment or to the informed consent, stating the reason and the actions needed to continue the study, with a subsequent decision by the LEC Chair;

to require changes to the documents relating to the proposed amendment or to the informed consent, stating the reason and the actions needed to continue the study, with subsequent consideration at an LEC meeting;

to suspend the study until further information is received;

not to suspend the study, as it has already been approved, but to request further information on the amendment and its impact on the conduct of the approved study;

not to approve the amendment, stating the reason, but to allow the study to continue as previously approved.

If the LEC approves the amendment, the LEC Secretary communicates the decision to the investigator. If the LEC does not approve the amendment, the LEC Chair immediately notifies the applicant in writing of the decision taken and the reasons for not approving the amendment.

If the LEC votes that any modifications to the documentation or to the protocol amendment are necessary, the LEC Secretary sends the applicant a written request to make the necessary changes and resubmit the documentation to the LEC.

The LEC Chair completes the decision form after the decision has been taken at the LEC meeting. The LEC Secretary keeps all forms and the minutes of the LEC meeting at which the decision was discussed and taken as official documents of the amendment review process.

6 **Notification of the decision** The LEC Chair notifies the applicant of the decision taken orally after the LEC meeting, and then in writing no later than 7 (seven) working days after completion of the review.

The LEC Chair must send an electronic version, or a copy by fax, of the application form (Appendix 1) bearing his/her signature and the date of approval to the LEC Secretariat within 1 (one) day, but no later than 3 (three) working days after the review the LEC Chair signs and dates the original of the form and returns it to the LEC Secretary no later than 5 (five) working days after the review the LEC Secretary appends to the document package number a letter corresponding to the amendment number.

the LEC Secretary signs and dates the original of the form.

7 **Notification of the applicant** The LEC Secretary sends the signed and dated amendment application form to the applicant for his/her records no later than 7 (seven) working days. The applicant must subsequently provide the LEC Secretary with a clean, i.e. corrected, version of the study document package (in accordance with the amendments made, without underlining or corrections).

8 **Document storage** The LEC Secretary places the originals of the completed documents and the corrected version of the documents in the appropriate folder

Appendix 1

Application form for an amendment to the study protocol

Protocol number:	Date of submission:

Title

Investigator's full name:

Study site:	Telephone
Date of approval:	Amendment number:

Rationale for the amendment:

Type of review required for the amendment:

☐ expedited (minor changes)

☐ full review (more substantial changes or changes that significantly increase the risk to study participants)

Signature: _____ (investigator's full name)

Date: «__»_____ _____

Comments: ☐ Expedited review ☐ Full review

Approved:

Date_____

LEC Chair

Completed by:

Date_____

LEC Secretary

Protocol number: ☐☐☐/☐☐-☐☐

Governing documents

1. Code of the Republic of Kazakhstan "On Public Health and the Healthcare System" No. 360-VI ZRK dated 7 July 2020. Link
2. Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 248/2020 dated 11 December 2020 "On approval of the rules for conducting clinical trials of medicinal products and medical devices, clinical laboratory testing of medical devices for diagnostics outside a living organism (in vitro), and the requirements for clinical sites, and for the provision of the public service Issuance of authorisation to conduct a clinical trial and (or) testing of pharmacological and medicinal products and medical devices". Link
3. Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 35 dated 4 April 2022, registered with the Ministry of Justice of the Republic of Kazakhstan on 12 April 2022 under No. 27526, on amendments and additions to Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ-248/2020 dated 11 December 2020 "On approval of the rules for conducting clinical trials of medicinal products and medical devices, clinical laboratory testing of medical devices for diagnostics outside a living organism (in vitro), and the requirements for clinical sites, and for the provision of the public service Issuance of authorisation to conduct a clinical trial and (or) testing of pharmacological and medicinal products and medical devices". Link
4. Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 310/2020 dated 21 December 2020 "On approval of the rules for conducting biomedical research and the requirements for research centres". Link
5. Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 181/2020 dated 4 November 2020 "On approval of the rules for assessing, within the framework of pharmaceutical inspection, the materials of preclinical (non-clinical) studies and the compliance of the conditions for conducting them with the requirements of Good Laboratory Practice (GLP) of the Republic of Kazakhstan and (or) the Eurasian Economic Union". Link
6. Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 151/2020 dated 23 October 2020 "On approval of the Regulation on the Central Bioethics Commission". Link

7. Order of the Acting Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 130 dated 20 December 2021 "On approval of the standard for the organisation of haematological care for the adult population of the Republic of Kazakhstan". Link
8. Order of the Acting Minister of Health of the Republic of Kazakhstan dated 4 February 2021 No. ҚР ДСМ-

15. Registered with the Ministry of Justice of the Republic of Kazakhstan on 9 February 2021 under No. 2216, "On approval of good pharmaceutical practices". Link

9. Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 304/2020 dated 21 December 2020. Registered with the Ministry of Justice of the Republic of Kazakhstan on 22 December 2020 under No. 21848, "On approval of the regulations on the clinical site, the clinic of a healthcare education organisation, the university hospital, the residency site and the integrated academic medical centre, and the requirements for them". Link
10. Order of the Minister of Health of the Republic of Kazakhstan No. ҚР ДСМ- 299/2020 dated 21 December 2020 "On approval of the rules for accreditation in the field of healthcare". Link
11. Standard ISO14155:2014№ "Clinical investigations. Good Clinical Practice". Link
12. Recommendation of the Board of the Eurasian Economic Commission No. 11 dated 17 July 2018 "On the Guideline on general issues of clinical trials". Link
13. Decision of the Council of the Eurasian Economic Commission No. 79 dated 3 November 2016 "On approval of the Rules of Good Clinical Practice of the Eurasian Economic Union". Link
14. Recommendation of the Board of the Eurasian Economic Commission No. 19 dated 3 November 2020 "On the Guideline on the application of biostatistics principles in clinical trials of medicinal products". Link
15. Recommendation of the Board of the Eurasian Economic Commission No. 18 dated 27 October 2020 "On the Guideline on safety pharmacology studies of medicinal products for human use".
16. Decision of the Council of the Eurasian Economic Commission No. 85 of 3 November 2016 "On the appro- of the Eurasian Economic Union". Link
17. Declaration of Helsinki of the World Medical Association, June 1964 (latest revision of 9 July 2018). Link
18. Standards and Operational Guidance for Ethics Review of Health-Related Research with Human Participants, World Health Organization (2011); and other international declarations and guidelines in the field of research ethics (WHO, ICH-GCP, UNESCO, CIOMS, CE).
19. Regulation on the Local Bioethics Committee (hereinafter- the LEC or the Committee) of HemCenter LLP, approved by Order No. I-02-44 dated 31 June 2023
20. Rules for conducting clinical trials and scientific research at HemCenter LLP, approved by Order No. I-02-47 dated 31 June 2023
21. Job descriptions of the Local Bioethics Committee of HemCenter LLP dated 31 June 2023.
22. Other regulatory legal acts of the Republic of Kazakhstan governing clinical trials.

Related documents

No.	type	title
Issl-3-2	SOP	Maintaining confidentiality in the work of the LEC
Issl-3-4	SOP	Process of Submitting an Application for Review of the Study Document Package (Study Protocol, etc.)
Issl-3-20	SOP	Maintaining the confidentiality of LEC documentation

List of changes

Rev. No. Date	Order No. List of changes **Change of the SOP form, corrections to terminology based on amendments/additions**

Document acknowledgement list

No.	Full name	Position	Date	Signature
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